

What is a community?

Definitions:

Broadly defined, a community is a collection of people who interact with one another and whose common interest or characteristics gives them a sense of unity and belonging.

Community is defined as an open social system that is characterized by people in a place who have common goals over time. The term is applicable to a variety of situations.

A community may also mean a group of people in defined geographical area with common goal and objective, interacting with one another.

“A community is a group of people with a common identity or perspective, occupying space during a given period of time, and functioning through a social system to meet its needs within a larger social environment.” (Higgs & Gustafson;1985).

According to WHO, "Communities" are groups of people that may or may not be spatially connected, but who share common interests, concerns or identities. These communities could be local, national or international, with specific or broad interests.

Anderson & McFarlane (2010); defined community in terms of a core dimension(people) and eight subsystems: (i) physical environment (ii) education (iii) safety and transportation (iv) politics and government (v)health and social services (vi) communication (vii) economics, and (viii) recreation.

Components of a Community:

A community may only be defined as a community if three critical components or defining characteristics are included – **people**, **place**, and **social interaction** or common characteristics, interests, or goals. All communities contain all three of these components.

1. **People** – population is the most obvious of the necessary components of community. However, the number of people included in the community depends on the other two critical components. A population can either be relatively small or large, such as an aggregate of 20 pregnant adolescents attending antenatal clinic, or a city of one million people respectively. The gender, age, religion, socioeconomic status, occupation, and race/ethnicity may be similar or different.
2. **Place**– communities traditionally were described in relation to geographical area. However, population aggregates such as the aged, the disabled, the poor, or any population in which the members share one or more common characteristics, goal, or interests are sometimes used to identify a community for assessment purposes.

Communities may therefore be defined by one of two designations: (i) geopolitical (spatial), or (ii) phenomenological (relational).

i). Geopolitical community is a spatial designation; that is, a geographical or geopolitical area or place. In geopolitical communities, place is designated by a geographical or political boundary. The people who live, work, learn, and play in the community constitute the population. In most suburban and urban geopolitical communities, the individuals know only some of the residents on a face-to-face, personal basis. In less densely populated rural areas, most people may know each other on a personal basis.

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Geopolitical communities are formed by either natural or human-made boundaries. A river, mountain range, or a valley may create a natural boundary. Human-made boundaries may be structural, political, or legal. Streets, bridges, or railway tracks may create structural boundaries, whereas cities, provinces or state lines create legal boundaries. Political boundaries may be exemplified by congressional districts.

ii). Phenomenological community is one in which its members have the sense of membership or feeling of belonging. Place is designated by sense of belonging among its members. Although all human communities exist in a physical place, the members of a phenomenological community are bound together by their interpersonal connectedness rather than by geography. An example is a population of people with common interests such as a common religious conviction or professional or academic interest. A sense of belonging occurs in phenomenological communities such as clubs, schools, gangs, centres for the seniors(aged), businesses, churches and other religious organizations.

Another example of a phenomenological community is a community of solution. This type of community has special significance for health planning. When health services are considered, the boundaries of each community are established by the boundaries within which a problem can be identified, dealt with, and then solved. A community of solution includes: (a) **a health problem shed** – that is, an area that has similar health problems and (b) **a health marketing area** – an area that has similar solutions to the problem or an adequate supply of health resources to meet the problem. All of the communities affected with a spillage of petroleum oil in a stream for instance, become the health problem shed, and all of the communities that join together and pool their resources to meet the need create a health marketing area.

3. Social Interaction or common interests, goals and characteristics – communities, similar to families, have their own patterned interaction among individuals, families, groups, and organizations; this interaction varies from community to community depending on needs and values. The various parts of communities' social system that interact include the health system, family system, economic system, political and educational system.

- In a geopolitical community, this interaction may go beyond talking to one's neighbour and may include interactions with agencies and institutions within the community. In a phenomenological community, this attribute is inherent, as it exists because of a common interest or feeling of belonging.
- Each of us lives in a geopolitical community, but we may be members of several phenomenological communities.

Goals of a Community

Goals of communities vary with the type of community, but in general, they are focused on maximizing the well-being of members, promoting survival, and meeting the needs of the community members. The community health nurse can assess the goals of the community by asking questions such as, "what is the purpose of the community?"

Characteristics of a Community

Characteristics are the physical, biological and psychosocial factors of the community. These characteristics are often referred to as demographics. Characteristics are usually not easily changed, or they change slowly.

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The **physical characteristics** of a community include: the length of time the community has been in existence; pertinent demographic data about the community's members (e.g., age, race, gender, education, ethnicity, housing, income, density of population etc.); and physical features of the community that influence behaviour.

a). The length of time the community has been in existence (i.e., the age of the community) has implications regarding stability, health care services, and needs. On one hand, a very new community may have few services simply because supply has not caught up with demand. While on the other hand, communities that have been in existence for a very long time may have many resources, or they may have resource that reflect past population needs but not the current needs. (if population shifts have occurred).

b). Pertinent demographic data such as age, race, gender, ethnicity, and density of the population have significant meaning in the planning of health care services. By looking at the age, gender and race of members of the community, the community health nurse (CHN) can make some inferences about the possible health care needs. Race is a factor in certain diseases; ethnicity is reflected in customs, beliefs, and values and may affect how the community addresses certain health practices. The CHN must understand these customs and beliefs when assessing needs and planning interventions. In some areas, the cultural and ethnic backgrounds of the population have become the basis for the community.

c). The type, condition, and amount of housing and density of the population are environmental factors that have implications for health. Crowded living conditions have long been associated with the increased transmission of some communicable diseases. Also, important to note is the condition of the housing and whether housing is available and financially accessible to people in the community. The type and condition of housing may say a lot about the resources and values of the people living in the community.

Physical features of the community can influence its behaviours. A community with fences around all houses demonstrates preference for privacy and may imply little social interaction. Other physical features such as living or working in a community with toxic substances may influence the level of health of the residents or workers.

Psychosocial characteristics – that affect the emotional tones of the community include religion, socioeconomic class, education, occupation and marital status. Some of the ways these characteristics may affect health behaviour include the following:

- Religion – beliefs may involve the use or nonuse of contraceptives, circumcision, living will and organ donation.
- Socioeconomic class – poverty reduces access to health care services and increases health risk.
- Education – higher educational levels are associated with higher rates of preventive health behaviours.
- Occupation – a person's livelihood may influence the risk of disease or injury. For instance, coal miners are prone silicosis and lung cancer, computer users to carpal tunnel syndrome, or sedentary white-collar professionals' coronary artery disease.

Points to Remember

- ✚ The function of any community includes its members' sense of belonging and shared identity, values, norms, communication, and supporting behaviors.

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- ✚ Some communities may share almost everything, while other communities (large, scattered and composed of individuals) may share only their common interests and involvement in certain goals. For example, individuals may belong to the U.S. military community, even though they live throughout the entire world; pregnant mothers scattered throughout a large city may emerge as a community through a common interest in their need for improved antenatal services etc.

Definition of Health According to WHO

The World Health Organization (WHO) defined health as “a state of complete physical, mental and social wellbeing and not merely the absence of disease or infirmity.”

This definition signifies the various parameters of man: Social, Physical and Mental status. There are three dimensions of health: physical, mental and social.

Physical: This deals with the practical engagement of the body in normal daily activities. It is the ability of an individual to do work, i.e. put into exercise muscles and bones to achieve work.

Mental: This is the ability to think and act rationally. It involves the appropriate use of the mind to do things objectively and reasonably. A mentally fit person has good perception of his surroundings/environment. This means that a mentally healthy person's intelligence, memory, learning capacity and reasoning faculty are normal. She/he has confidence in herself/himself, and realizes her/his capabilities. In addition, her emotional reactions are proportional to the intensity of the stimulus concerned.

Social wellbeing: This is the innovative aspect of health. It deals with the ability to create and maintain cordial relationships, of families, friends, neighbours etc. It also involves creating and developing safe and healthy environments. A socially healthy person contributes constructively to the progress of society, i.e. pays her taxes, behaves herself in public, participates in community functions etc.

REMEMBER: “A lack or deviation in any of these characteristics or dimensions describes the absence of normal health.”

What is Public Health?

Public Health is the Science and Art/practice of health promotion, disease prevention and efficient prolongation of life through organized societal effort.

It is a science because it is made of a body of scientific knowledge accumulated through research. It is the practice undertaken by the health care provider, individual and families in the community for the purpose of healthful living. The art of preventing disease is achieved by improving environmental sanitation, providing health education and curative services. This involves the participation of official agencies in promoting such measures.

In the definition are five key words that are discussed as follows;

Public health as a science-this refers to the body of knowledge, a series of facts or a set of principles. For example, it is a known fact that chlorine when applied to water

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kills germs present in it. This fact in addition to many other proven facts makes this knowledge scientific.

Public health as an art-art refers to the translation of theoretical knowledge into practice or the conversion of principles to procedures. In the above example the application of chlorine to water wells or water meant for drinking is an art, i.e. fact is translated into practice.

Preventing diseases-this refers to the three disease prevention levels, which are primary secondary and tertiary levels of prevention. Disease refers to all kinds of infections, injuries, defects and disorders that affect people.

Prolongation of life means the prevention of premature death, whether due to disease, wars, violence or natural disasters.

Organized societal effort stresses the fact that prevention of diseases and promotion of health of people cannot be undertaken by a single individual. It requires the cooperate effort of willing and capable people to plan, formulate policy, and implement activities aimed at improving their health.

In addition to this definition **public health deals with all aspects of human activity that affect health**. It concerns with the **promotion and maintenance of health among groups of communities or nations**. This definition recognizes the fact that, factors predisposing to ill health are not only due to a lack of knowledge, but rather social problems that come in the way of health practices.

Definitions of Nursing

Nursing is defined by various nursing theorists explaining their concepts about nursing. However, ICN adopted definition was that presented by Virginia Henderson. She stated that “Nursing is the assistance given to a person whether sick or well, in those activities contributing to health or its recovery or leading to a peaceful death, which the individual would have performed unaided if he had the necessary will, strength or knowledge”. This definition is not limited to an individual it also applies to families and communities.

The American Nurses Association also gave a scientific definition of nursing which stated that: **“Nursing is a science that deals with the identification and management of human responses to actual or potential health problems.”**

Nursing focuses on human responses to actual or potential problems, whereas Medicine focuses on identifying and managing health problems.

What are these human responses? These are what concerns the nurse otherwise referred to as: Nursing Problems or Nursing Diagnoses. Whichever definition of nursing that exists, the main concern is caring for people in need to address the activities of daily living as stated by Virginia Henderson.

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Public Health Nursing

Public Health Nursing is a specialized field of nursing which utilizes the science and practice of public health in Professional Nursing practice.

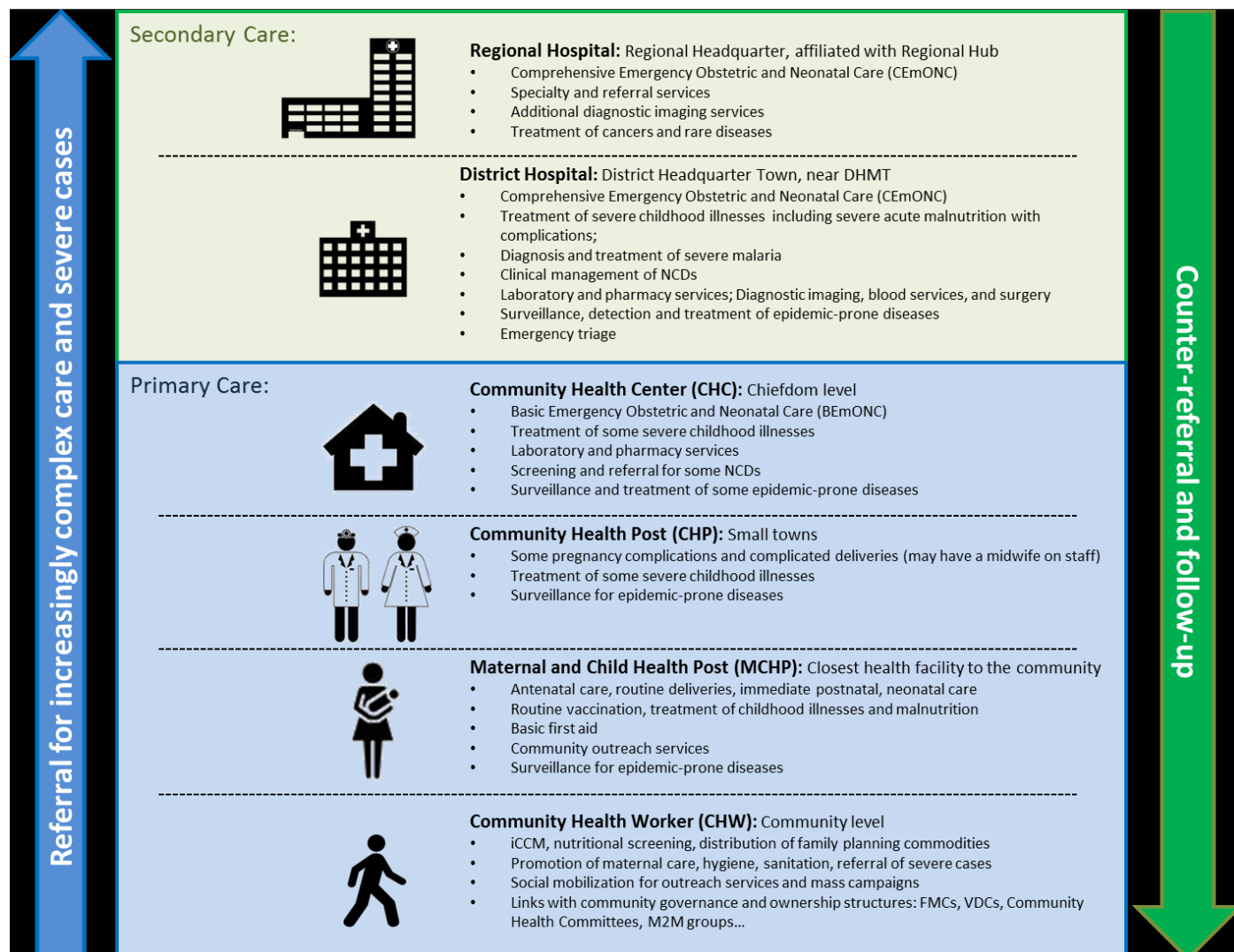
Characteristics of Public Health Nursing:

1. It is population oriented. It focuses on caring for individuals and families in the community
2. It is applicable in all community setting
3. It aims at disease prevention and health promotion
4. It utilizes the principles of interdisciplinary approach i.e., collaborating with other allied professionals in health care at all levels.
5. Although it is a specialty in nursing, it still utilizes the nursing approaches to care for needy individuals, families, groups and communities

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LEVELS OF HEALTHCARE SERVICE DELIVERY IN SIERRA LEONE



1. PRIMARY CARE SERVICE DELIVERY

It is the first level of contact of individual, family and community with health system. It is most effective and close to the people. Primary health care addresses the main health problems in the community, promotes health and wellness and seeks to prevent injuries and illness. It's about more than delivering health care services. It's about creating the conditions that help people to become and stay healthy and well. It's also about extending the reach of health care providers into communities.

Primary health care is delivered in four levels of increasing clinical skill and capacity to handle complications (three levels of health facilities and community level).

Primary health care facilities are referred to as Peripheral Health Units (PHUs). There are three levels of PHUs which are recognized and standardized: Maternal and Child Health Post (MCHP),

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Community Health Post (CHP), and Community Health Centre (CHC). Each level has clearly defined functions. The buildings, equipment, drug and supplies, and staffing are specified to meet the functions. PHUs have preventive and curative functions and are supposed to be open 24 hours, or have a staff on-call for off hours patient needs. MCHPs and CHPs have beds only used for observation. Patients requiring further supervised care are referred to the CHC or hospital. CHCs, where a wider range of more complex services are offered, admit cases referred from the lower levels. A limited range of preventive and basic curative services are also available directly at community level (outside of health facilities but with linkage to PHUs through supervision, reporting, and supply chain management).

COMMUNITY

The 2010 BPEHS described a wide variety of fragmented community-based health programs, and expressed the need to harmonize roles through a Community Health Worker (CHW) policy. This policy was developed in 2012, and harmonizes the various community volunteers including TBAs, social mobilizers, community drug distributors, blue flag volunteers, etc. under one umbrella with standardized selection criteria, roles, and training requirements. The policy further outlines a specific package of services that can be offered by trained CHWs at community level. These include:

- Community sensitization and advocacy
- Home visits to promote preventive care, hygiene and sanitation, ANC, birth preparedness and skilled delivery, PNC, nutrition, and other essential services
- ICCM consisting of diagnosis and treatment for simple diarrhea, pneumonia, and malaria (including provision of ACTs and antibiotics as needed)
- Screening and referral for acute malnutrition
- Defaulter tracing
- Vital events reporting
- Reporting of outbreaks; and
- Some family planning services.

The services listed in the basic package below are more extensive than those described in the CHW policy and training manual. They also include supplementary services which could be added on to their workload as and when appropriate, or that may be provided by other community-level service providers or as outreach from the MCHP. As the CHW policy, strategy, and training manual are revised in the early recovery phase, the community level service package may be expanded to include more or all of the services listed below.

CHWs are the main staffing cadre at community level. The 2012 CHW policy provides guidance that there should be one CHW per between 100 and 500 population (20 to 100 households). This is variable depending on the size of the community – each village should have at least one functioning CHW – as well as population density, i.e. urban versus rural context. For the purposes of human resource projections, it is estimated that approximately 16,000 CHWs are needed for full coverage (1:375 based on a 2015 projected population of 6,014,804). A national CHW mapping

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in 2014 identified 13,000 functional CHWs in the country being supported by various implementing partners. This mapping will be updated in 2015 to account for changes in workforce due to the Ebola outbreak and routine attrition.

CHWs are required to undergo a 10-day standardized training in order to be certified and recognized within the health system. Community volunteers who previously worked under other titles are required to undergo the comprehensive training in order to work under the title of CHW4. While CHW programs are currently managed by non-governmental implementing partners (international and national NGOs), there remains a need to institutionalize this cadre under the MoHS as part of the national health workforce. Achievement of this goal is planned under the NHSSP 2015-2020.

A. MATERNAL AND CHILD HEALTH POST (MCHP)

MCPH is the most peripheral level of PHU. An MCHP should ideally serve a population of 500 to 5,000 within a 5 km (3 miles) radius of the facility. Staffed by MCH Aides, MCHPs are often the first facility level of contact for patients. As the name suggests, services in the MCHP focus primarily on antenatal care, safe and skilled deliveries (without complications), post-natal care, and child health services including EPI, nutrition, and IMNCI. As they are the closest facility to the community level, MCHPs have a close relationship with CHWs and an active role in implementing outreach campaigns including Maternal and Child Health Week, National Immunization Days (NIDs), and bed net distribution campaigns. MCHPs do not have pharmacies but MCH Aides are qualified to dispense free health care drugs and other basic medicines.

B. COMMUNITY HEALTH POST (CHP)

CHPs are usually situated in a smaller town, and serve a population of 5,000 to 10,000 or more within 8 km (5 miles) radius of the facility. These posts have similar functions to the MCHP with added curative functions. An SECHN or Community Health Assistant (CHA) typically serves as the in-charge in CHPs. Looking forward, as the availability of trained SECHN Midwives increases, CHPs will be staffed by a trained midwife and have the capacity to handle some complicated births and obstetric and neonatal emergencies.

C. COMMUNITY HEALTH CENTER (CHC)

CHCs are usually situated in the chiefdom headquarter town or in a well-populated area with a catchment population of 10,000 to 30,000 or more within 15 km (10 miles) radius of the facility. The CHC has preventive and curative functions. It offers the most complex and skilled services within the primary care level of the health system. Since 2010, the Ministry of Health and Sanitation has been working towards facility readiness goals to certify 65 CHCs for Basic EmONC (BEmONC) – five CHCs per district. Ideally, all CHCs should eventually be able to provide a full BEmONC package. According to the December 2014 FIT Assessment, no CHCs currently meet the full requirement, but improvement towards this goal is an objective of the Health Sector Recovery Plan and NHSSP 2015-2020. The CHC is the first level of health facility that has both a

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laboratory and a pharmacy, and thus has much higher diagnostic and treatment capacity than MCHPs and CHPs.

The in-charge in a CHC is a Community Health Officer (CHO), supported by a team consisting of a CHA, SECHNs, Midwives, MCH Aides, and other clinical and support staff. The CHC supervises the lower levels of care, including CHWs, MCHPs, and CHPs within its catchment area.

OVERVIEW OF PRIMARY HEALTH CARE (PHC).

“All people, everywhere, have the right to achieve the highest attainable level of health.” This is the fundamental premise of primary health care.

What is PHC?

It is the “first” level of contact between the individual and the health system in which essential health care is provided. A majority of prevailing health problems can be satisfactorily managed. It is the healthcare closest to the people that the primary health centres provide.

Primary health care (PHC) became a core policy for the World Health Organization with the Alma-Ata Declaration in 1978 and the ‘Health-for-All by the Year 2000’ Program. Primary health care is about health at all ages. It involves prevention, health promotion, treatment, rehabilitation and palliation. This approach empowers people and communities to choose healthier lifestyles, prevent diseases, and access early detection, treatment and recovery services.

Primary health care **is not** about building specific health facilities. It is an overall approach to strengthening national health systems to respond to people’s essential health needs more effectively.

PHC According to the Alma Ata Declaration

Alma Ata is a name that has become synonymous with one of the great public health movements of history – that is the quest for equity in health that WHO expressed as its goal which is “Health for All” by the 2000. PHC is usually associated with the declaration of the 1978 International Conference in Alma Ata, Kazakhstan (known as the “Alma Ata Declaration”). The Alma Ata declaration of 1978 emerged as a major milestone of the 20th century in the field of public health and it identified PHC as the key to the attainment of the WHO’s goal of “Health for All.”

According to the Alma Ata declaration, **“Primary health care is essential health care based on practical, scientifically sound, and socially acceptable methods and technology made universally accessible to individuals and families in the community through their full participation and at a cost that the community and country can afford to maintain at every stage of their development in the spirit of self-reliance and self-determination.”** The definition means that Primary Health Care is basic health services provided to every individual at acceptable quality, distance and cost within the limit of resources the community can afford to maintain. The services should be provided with the collaboration of the people and generally without discrimination.

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The Conference of Alma Ata, which took place in September 1978, made PHC a central concern for the WHO and many health workers worldwide. The commitment to global improvements in health, especially for the most disadvantaged populations, was renewed in 1998 by the World Health Assembly. This led to the 'Health-for-All for the twenty-first Century' policy and program, within which the commitment to PHC development is restated.

The Primary Health Care approach integrates at the community level all the factors required for improving the population's health status. PHC is for all, especially the needy, and regardless of social and economic status, every individual in the nation must have access to health care. The services should be acceptable to the community, and the community must be actively involved. The health services must be effective, preventive, promotive and curative. The services should form an integral part of the country's health system. The programme must be efficient and multi-sectorial because health does not exist in isolation.

Why is PHC Important?

The world has committed to making "health for all" a reality, and primary health care is one of the best tools we have for achieving that goal. There is a growing body of evidence in countries that support economic arguments in favour of increasing investment in primary health care. It has been proven that health systems with a primary healthcare-based foundation result in:

- improved clinical outcomes
- increased efficiency
- better quality of care
- enhanced patient satisfaction.

Principles of Primary Health Care

Key principles of Primary Health care are derived from the Alma Ata Declaration.

Equitable and accessible Distribution: Health services should be equally accessible to the whole population, including rural communities and urban slums.

Community Involvement: The community should effectively participate in planning, decision-making, implementation, monitoring and supervision of activities related to the provision of health care delivery services.

Focus on Prevention and Control: Health promotion through Health Education and Disease Prevention and Control through community programmes as well as curative services should be encouraged.

Appropriate Technology: The methods and materials used should be acceptable to the community, relevant to the major health problems and within the country's economic resources.

Multi-Sectoral Approach: Health care is part of development. It includes nutrition, water supplies, sanitation, education, and other line ministries that contribute to the general rise in communities' standard of living.

Sustainability: Sustainability is basic to improvement. It is how we accumulate benefits and thus improve ourselves.

Constant Monitoring: All programmes should constantly monitor and supervise their workers to maintain and improve the level of their positive effects on the community

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Decentralization: It is the sharing and transferring of power and decisions away from the centre to the periphery. It brings decisions closer to the communities served and the field-level providers of services. It leads to greater efficiency in service provision.

Components of Primary Health Care

There are traditionally eight components (elements) of primary health care. However, more had been included:

Immunization: An increasing number of infectious diseases can be prevented by vaccinations; for example, measles, Meningitis, pertussis, Tuberculosis, Yellow fever etc.

Maternal and Child Care: Pregnant women and women of childbearing age (15-49 years) are the target group for special care. Children under 5 years of age are also vulnerable to childhood killer diseases. Maternal and child health clinics are established in Nigeria to care for these groups.

Provision of Essential Drugs: The most vital drugs should be available and affordable at all levels.

Food and Nutrition: The family's food should be adequate, affordable and balanced in nutrients.

Health Education: The community should be informed of health problems and methods of preventing and controlling them.

Treatment of Minor Illness and Injury: The community health worker should make adequate provision of curative services for common ailments and injuries.

Provision of Safe Water Supply and Sanitation: A safe water supply and the adequate disposal of wastes are vital for health.

Prevention and Control of Locally Endemic Diseases (Vector and Reservoirs): Endemic infectious diseases can be regulated through the control or eradication of vectors and animal reservoirs.

In addition to the original eight components of PHC, a number of key health issues are today considered as constituents of PHC. They include:

Dental Health: Dental and oral health is an essential part of people's overall health and well-being. Dental health has been given less priority because of other health problems which are life-threatening and communicable. According to W.H.O., dental health problems are of growing concern to most African countries.

Mental Health: In low-income countries where morbidity and mortality due to malnutrition and preventable infectious diseases are very common, mental disorders, which are not regarded as life-threatening problems, are considered to be insignificant and unworthy of attention. Since Sierra Leone is one of the poorest countries in the world, providing high-standard mental health services to needy people is not an easy task, and the situation reflects the lack of attention indicated above.

Occupational Health

Use of Traditional Medicine

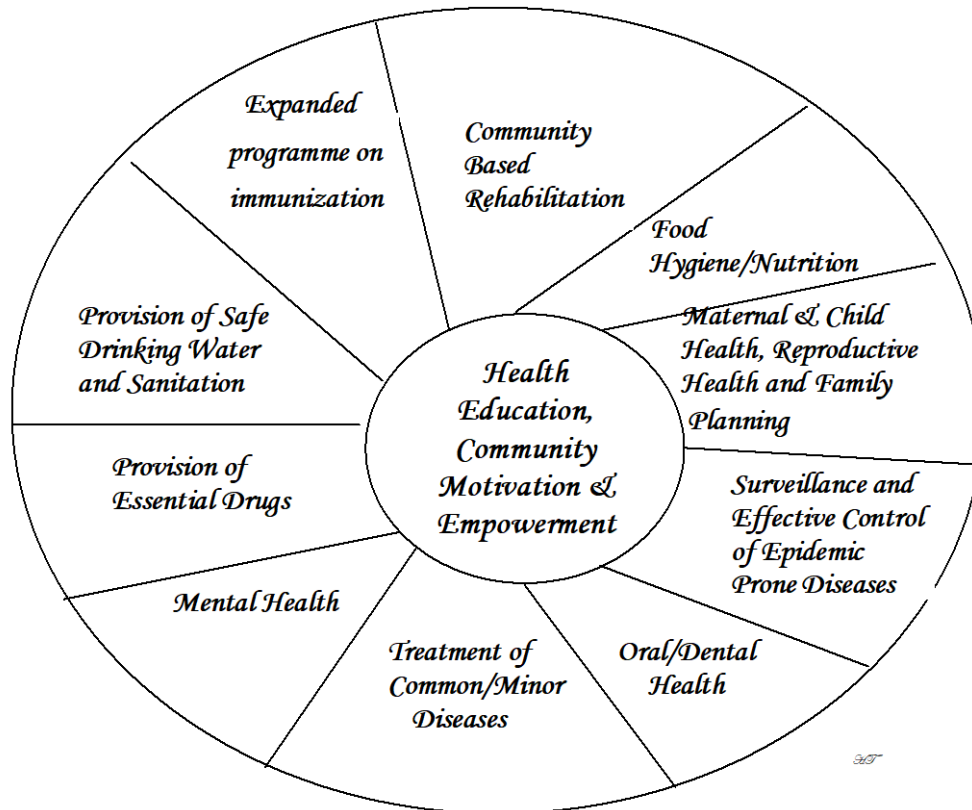
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HIV/AIDS

See below the diagrammatic presentation of PHC components:

The main elements of Primary Health Care



Remember!!

The existing health services provided by Government, private and NGOs are already performing many PHC activities. Primary Health Care is not a new concept, nor is it something separate from existing services. This statement is fundamental to the understanding of Primary Health care. Primary Health Care is essentially an improvement and expansion of activities already being done in Health Promotion, Prevention and Control and effective Community Involvement combined with an effort to reach more people

BACKGROUND OF PRIMARY HEALTH CARE IN SIERRA LEONE

Sierra Leone was a signatory to the famous declaration formulated at the WHO/UNICEF conference at ALMA ATA in 1978 (Marcella Davies of Sierra Leone read the consensus of 134

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countries). The need to make this declaration operational and implement it was underlined in the health policy document of June 1993.

Thus, there has been a determined effort to implement the ideals the country has subscribed to by signing. The following are some of the more important steps taken since 1978.

1. A series of intersectional, regional and national workshops to discuss the concepts of Primary Health Care were held in Kenema, Bo, Makeni and Freetown during 1982/83. These workshops were attended by delegates from the Ministries of Education and Agriculture as well as from the then Ministry of Health and Social Services
2. The Government has encouraged the development of several projects, which support the PHC approach within the country, funded and organized by Non-Governmental Organizations (NGOs). The two largest projects were those at Bombali, partly supported by UNICEF and a church agency (Meserior- Catholic church organization) and WHO and the second was the GTZ-Funded Bo-Pujehun project in the Southern Province.
3. The Government/World Bank Health and Population Project (HPP) in 1986 supported PHC in Kenema, Moyamba, and Tonkolili Districts. These were maintained by the collaborative inputs of the Government and UNICEF. Plan International supported the Rural Western Area Primary Health Care Programme in 1993. In furtherance to government commitment to PHC the European Union (EU) is supporting PHC in Kailahun, Kambia, and Pujehun Districts respectively.
4. To unify the approach, the national guidelines concerning the development and implementation of Primary Health Care were developed and embodied in THE NATIONAL OPERATIONAL HANDBOOK FOR PRIMARY HEALTH CARE, which was first published in 1987.
5. Following the publication of these guidelines, a further series of national workshops were held to discuss the detailed implementation of Primary Health Care within the Ministry of Health facilities and services. District Medical Officers, Public Health Sisters and Environmental Health Superintendents, attended these workshops. Special attention was given to the development of Peripheral Health Units and training of auxiliary staff.
6. Between 1986 and 1989, substantial support has been given and efforts made in promoting PHC components, notably immunization and essential drugs availability. These two named components have achieved a national coverage and represent a firm foundation for the consolidation and expansion of the District based PHC approach referred to in 2 above. A five-year programme proposal to proliferate community-based MCH/PHC nationally, between 1989 and 1993, was finalized by the Ministry of Health, in collaboration with UNICEF and WHO. This proposal, inter-alia, sought to develop the successful introduction

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of the community financing of PHC-component services and achieve the self-sustainability of PHC, which had not been fully taken into consideration previously.

7. To strengthen and enhance PHC implementation, Ministry of Health is now designated the Ministry of Health and Sanitation to emphasize the Environmental Sanitation aspect of its functions. Further to the previous restructuring of its administration, apart from the Director General Management Services, a Directorate of Primary Health Care was created to oversee the following thirteen (13) programs whose responsibilities focus on particular aspects of PHC.

To further strengthen the Directorate of PHC, a new Directorate has been created to oversee disease prevention and control programmes. These include:

- Malaria control programme.
- Leprosy/Tuberculosis programme.
- HIV/AIDS.
- Neglected Tropical Diseases (NTDs)
- Expanded Programme on Immunization
- Integrated Management of Neonate and Childhood Illnesses.

8. The Health Policy was formulated in 1993.

9. The National Health Action Plan was put in place in 1994 and is now being implemented.

10. Planning at the micro level by various managers within the Directorates is ongoing and regular.

11. Detailed district plans are regularly developed.

12. All plans are formulated according to the following:

- What must be done?
- Who must do it?
- Within what period of time
- With what resources?
- How is it evaluated?

THE BAMAKO INITIATIVE

The 1987 Bamako Initiative was a conference of African Health Ministers held in Mali sponsored by UNICEF and WHO. The conference was summoned due to a realized gap in resource provision and implementation strategies of PHC in developing countries. Health workers lack resources to provide adequate health services, which led to a bypass of conventional treatment by the people to the use of herbs.

The objectives of the conference include advocacy for increase donor support and an increase in government budget on health. The conference also advocated for user fees and the management of such fees by community people. At the end, the conference succeeded in increasing immunization coverage, access to health and provision of resources.

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In Sierra Leone the following documents are essential for the effective implementation of PHC: the National Health Policy; National Health Action Plan; the Bamako Initiative; Supervisory Protocol; and Handbook on the Rational use of drugs.

They together constitute a body of documents indispensable for the effective implementation of Primary Health Care

2. SECONDARY CARE SERVICE DELIVERY

Secondary care is provided in district and regional hospitals. Although hospitals are primarily secondary care institutions, in most cases they also provide many primary care services to the population in their immediate surroundings, including ANC services and under-5's clinics.

District hospitals receive referrals from primary care facilities, and accept walk-in patients directly.

District Hospital

District hospitals are open 24 hours, and serve the whole district, with a catchment population of about 500,000. The staff are usually organized in three shifts in order to provide appropriate medical, midwifery and nursing coverage at all times. The district hospital provides some primary health care to its immediate catchment population, secondary medical care, emergency medical services, and comprehensive emergency obstetric and neonatal care (CEmONC) including general surgical and surgical obstetric care. To manage this, it has an operating theatre, an extensive laboratory with safe blood transfusion services, an X-Ray machine and ultrasound. District hospitals generally have more than 100 beds, and are staffed with doctors, midwives, nurses, nutritionists, and other categories of health staff in line with the HRH staff requirements.

3. TERTIARY CARE SERVICE DELIVERY

Sierra Leone does have a limited number of hospitals (all located in the capital, Freetown) that are considered to be tertiary level facilities – Connaught Hospital, Ola Daring Children's Hospital, Princess Christian Maternity Hospital, Lakka, Kissy Mental Hospital, and Jui. Some of these facilities do provide a limited range of primary care services. For example, PCMH runs a routine ANC clinic for women who live in close proximity to the hospital and for whom the hospital is the nearest health care facility.

Regional Hospital

Regional hospitals provide further specialized care, and accept referrals of complicated cases that cannot be managed at the district hospital. Regional hospitals are situated within the provincial capital towns (Kenema for the East, Bo for the South, and Makeni for the North). In these towns, they are an expansion of the district hospital. In addition to providing the full package of services available in district hospitals, regional hospitals are additionally staffed with specialists that are not available in all of the districts, and are equipped with more complex equipment. They have higher capacity to handle complex infectious diseases, emergency services, surgery, and radiology. The Health Sector Recovery Plan includes the establishment of three Regional Hubs, which will be attached to the regional hospitals. As part of the overall strengthening of the health workforce, the three regional hubs will serve as centers for excellence that include a critical mass of specialists

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who will be deployed as needed to districts in their coverage area. In 2015, the main focus will be the establishment and strengthening of these hubs. Moving forward into the recovery and resilience-building phases, the regional hubs will build up their capacity to support regional referrals and training needs. Hubs will concentrate health expertise to enable skills transfer, improvements in patient safety, mentoring and coaching for quality of care, and the creation of a continuum of care network. Quality of care guidelines will be developed including patient safety with a focus on IPC, along with improved surgical, medical, paediatric and obstetric/gynaecological services, for hospitals, PHUs and community level service delivery.

District Health System

The Ministry of Health and Sanitation (MoHS) is responsible for all policy direction and is organized into two main divisions at the central level: Medical service and management services. At the district level, the same two-division approach is adopted: district health services and the district health management both under the leadership of District Medical Officer (DMO)

The management service which is the administrative wing of the ministry has four units: Financial Resources, Internal Audit, Human Resources and Support Services. These are further sub-divided into smaller units overseen by managers. The Directors and Managers under the Chief Medical Officer are responsible for the preparation and implementation of the central level technical and support programme activities.

At the District level, the District Health Management Team (DHMT) coordinates, oversees and supervises all Public Health Care activities delivered through the Health Care Delivery system. The team is headed by the DMO who reports to the Director of PHC and to the programme Manager for Child Health/EPI activities at central level. Within each district, there are Peripheral Health Units (PHUs) and a Governmental District Hospital. The DHMT is responsible for managing the implementation of District Health Plans. With the decentralization of health service delivery, the local Councils now provide funding and other forms of support for Public Health activities at district levels.

Composition of the DHMT

The District Medical Officer is the Administrative Head and is responsible for convening regular meetings of the team which comprises the following:

- The Medical Officer or Medical Superintendent or Specialist in charge of the district Hospital
- The District Health Sister (DHS)
- The District Environmental Health Superintendent (DPHS);
- The Hospital Matron
- The Monitoring & Evaluation Office
- The Health Education Officer
- The District Pharmacist
- The Health Administrator or Hospital Secretary
- The WATSAN Co-coordinator.

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- Representative of Community Health Officers
- Finance Officer
- District Operation Officer (D.O.O.),
- District Social Mobilization Officer,
- Birth and Deaths Registrar
- MCH Aides training Coordinators.
- Disease Surveillance Officer

Other personnel may be co-opted into the team as and when their expertise is required. These include, Specialist Physician, “Head of Health Training schools, or persons from International Agencies or Missions who work in the health field. The District Health Management Team needs to meet at least once every month and or more often when necessary e.g. when training programmes are underway or at times of emergency, like epidemics.

Roles and Functions of DHMT in PHC

- The District Health Management team are responsible for the overall implementation of the PHC at all levels of health service delivery as well as coordination of all players within the district of operation. The DHMT will co-opt members from other relevant government departments including the local government, civil society, community representatives among others to ensure inclusiveness and accountability. An expanded district PHC technical group chaired by representative from DHMT but including other sectors will be constituted to provide technical guidance to the DHMT in the implementation and monitoring of the strategy.
- The DHMT takes responsibility for the planning, organising, implementing, monitoring and evaluating the primary health care activities. They should meet at regular intervals to assess progress based on evidence, plan, organize, implement and administer delivery of primary health care services, particularly the high impact interventions, focusing on priority issues such as the reproductive maternal neonatal child and adolescent health care as part of the basic package of essential health services. They supporting PHC governing structures including the district TWG. The team should lead and coordinate stakeholders (programs, NGOs, partners, health related sectors) involved in health care delivery in the district, to ensure optimal engagement of everyone for health. DHMT take responsibility for the MCH clinics situated at District Headquarters.
- DHMT should ensure collection, collation, analysis and use of data relating to PHC services within the District and use it to monitor, evaluate, progress, review priorities and plan, based on available routinely collected data. It ensures surveillance and prompt notification of all epidemic prone diseases and other notifiable diseases and take prompt action to control the outbreaks of such diseases.

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- The District Health Management Team needs to make a brief “overview” of the main health problems of the District, based on available routinely collected data, to be used for decision making and planning. In this way the health staff in clinics and PHUs are guided into ways and means of tackling the problems in partnership with the community and other stakeholders. For example, without high immunization coverage with potent vaccines, measles epidemics and death will remain a threat. There is need to continue improving early detection and treatment of tuberculosis, and many other chronic conditions. The District Health Management Team should ensure that all the PHU facilities in the district conform to the standard requirements. This rural Health Service is the basic infrastructure through which all Primary Health Care operates and through which all new planning innovations are effected.
- A major task of the District Health Team is to give support, supervision and training in order to bring each unit up to its full functional potential. Organize the number and distribution of the Peripheral Health Units within the District to meet the requirements of population distribution and density i.e. to make PHC universally accessible. DHMT equip and supervise the PHU staff and train and post the staff Revitalize the existing network of health units by providing equipment, drugs and supportive supervision. Train staff of the PHUs in Primary Health Care activities and arrange regular workshops for this purpose.
- The DHMT should facilitate community engagement in health care and the development of Chieftdom, and Village Development Committees as linkage structures between households and the health system, the health facilities. DHMT should budget and solicit funds based on rational budgeting and monitor expenditure to ensure accountability for PHC services. Deployment of Human Resource based on volume and type of work, and ensure optimum performance through effective communication, teamwork and performance management. They should ensure timely procurement of equipment and drugs for health facilities in the District. Ensure surveillance and prompt notification of all epidemic prone diseases and other notifiable diseases and take prompt action to control the outbreaks of such diseases.
- Collect, collate and analyze information relating to health and the health services within the District and use it to monitor, evaluate and plan. Monitor the health situation and the health services of the District. Improve the capacity and services of the District hospital to enable it to cope with the increased referrals resulting from improved PHC activity in the District. DHMT is responsible for posting all categories of staff within the District. Coordinate the activities of all health-related NGOs in the District. The average District may have 50 or more Peripheral Health Units and well over 100 technical staff, plus labourers, porters and other minor grades staff. Peripheral Health Units are the delivery points of

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Primary Health care. The support, equipping, maintenance and supervision of these units together with staff postings, arrangements for staff training and collection of records and reports, represent a major administrative task. It is clear that the District Management Health Team needs an administrative base. Mobilize financing.